Ibà — Fever-Triage Copilot

Clinical Review: 60 Synthetic Vignettes

Every case below is synthetic (no real patient data). Each one shows the presentation the app was given, the triage level and differential our build team set as the gold answer, and the guideline citation behind it. Please mark whether you agree with the gold answer for each case, and write a correction where you don't.

Reviewer: Dr. Oluba O. Amos, PharmD (@AmazingAmos)
Date reviewed: _____

How to mark a case: circle Agree or Disagree, and if Disagree, write the correct triage level and/or differential in the Correction line. Ibà escalates rather than under-triages wherever the guideline doesn't fully settle a question — flag anywhere that trade-off looks wrong for real PHC use, not only outright errors.

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Severe malaria

sm-01 *(Lagos)*

Man 40 years, fever 3 days, RDT positive, confused and talking nonsense since morning

Gold triage level: **Refer now**

Danger signs: lethargy_or_unconsciousness

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer.*

Agree / **Disagree** Correction: _____

sm-02 *(Oyo)*

Woman 32 years, fever 2 days, RDT positive, too weak to stand, cannot sit without support

Gold triage level: **Refer now**

Danger signs: prostration

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer.*

Agree / **Disagree** Correction: _____

sm-03 *(Kano)*

Child 4 years, fever 2 days, RDT positive, had convulsions twice today

Gold triage level: **Refer now**

Danger signs: convulsions

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer. WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / **Disagree** Correction: _____

sm-04 *(Kaduna)*

Man 28 years, fever 4 days, RDT positive, yellow eyes and dark cola coloured urine

Gold triage level: **Refer now**

Danger signs: jaundice, dark_urine

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer.*

Agree / **Disagree** Correction: _____

sm-05 *(Enugu)*

Child 2 years, fever 3 days, RDT positive, very pale palms, breathing fast and hard with chest indrawing

Gold triage level: **Refer now**

Danger signs: respiratory_distress, severe_pallor

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer. WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

sm-06 *(FCT)*

Woman 26 years, 6 months pregnant, fever 2 days, RDT positive, vomiting everything she drinks

Gold triage level: **Refer now**

Danger signs: vomiting_everything

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer. Pregnancy increases risk.*

Agree / Disagree Correction: _____

sm-07 *(Kwara)*

Man 55 years, fever 3 days, RDT positive, bleeding from the gums, very weak

Gold triage level: **Refer now**

Danger signs: bleeding

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer.*

Agree / Disagree Correction: _____

sm-08 *(Rivers)*

Child 18 months, fever 2 days, RDT positive, unconscious, not responding to voice

Gold triage level: **Refer now**

Danger signs: lethargy_or_unconsciousness

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer. WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

sm-09 *(Kano)*

Pikin 2 years, hot body 3 days, e dey convulse, e no fit drink, RDT positive

Gold triage level: **Refer now**

Danger signs: convulsions, unable_to_drink_or_breastfeed

Top differential: severe malaria

Flagged as tricky: *Pidgin*

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer. WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

sm-10 *(Lagos)*

Man 35 years, fever 5 days, RDT positive, difficulty breathing, passing very little dark urine

Gold triage level: **Refer now**

Danger signs: respiratory_distress, dark_urine

Top differential: severe malaria

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer.*

Agree / Disagree Correction: _____

Paediatric danger signs

pd-01 *(Oyo)*

Baby 8 months, fever 1 day, not able to breastfeed since morning

Gold triage level: **Refer now**

Danger signs: unable_to_drink_or_breastfeed

Top differential: severe febrile disease, sepsis, malaria

Guideline rationale: *WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

pd-02 *(Lagos)*

Child 18 months, fever 1 day, vomits everything, RDT positive

Gold triage level: **Refer now**

Danger signs: vomiting_everything

Top differential: severe malaria

Guideline rationale: *WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

pd-03 *(Enugu)*

Pikin 1 year, hot body 2 days, e just dey quiet, e no dey play, e dey sleep sleep

Gold triage level: **Refer now**

Danger signs: lethargy_or_unconsciousness

Top differential: severe febrile disease, sepsis, malaria

Flagged as tricky: *single subtle danger sign, Pidgin*

Guideline rationale: *WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently. Lethargy described subtly in Pidgin.*

Agree / Disagree Correction: _____

pd-04 *(FCT)*

2-year-old with fever 1 day, mother says he is unusually sleepy and hard to wake

Gold triage level: **Refer now**

Danger signs: lethargy_or_unconsciousness

Top differential: severe febrile disease, sepsis, malaria

Flagged as tricky: *single subtle danger sign*

Guideline rationale: *WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

pd-05 *(Kwara)*

Child 3 years, fever 1 day, had one convulsion at home, now awake and alert, RDT positive

Gold triage level: **Refer now**

Danger signs: convulsions

Top differential: severe malaria

Flagged as tricky: *danger sign resolved on arrival*

Guideline rationale: *WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently. A convulsion during this illness is a danger sign even if now alert.*

Agree / Disagree Correction: _____

pd-06 *(Rivers)*

Child 4 years, fever 2 days, cough, chest indrawing, grunting

Gold triage level: **Refer now**

Danger signs: respiratory_distress

Top differential: pneumonia, severe pneumonia

Guideline rationale: *WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently. WHO IMCI cough/difficult breathing (p.6).*

Agree / **Disagree** Correction: _____

Suspected Lassa fever

la-01 *(Ondo)*

Adult man 35 years, fever 5 days, RDT negative, took coartem for 3 days but no improvement, headache and sore throat

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Flagged as tricky: *regression from live CLI run 2026-09-22*

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Iba policy (pending clinician review, docs/CLINICAL_REVIEW.md §1): case definition met in an endemic state -> Refer now; partial features -> Refer within 24h.*

Agree / **Disagree** Correction: _____

la-02 *(Ondo, live outbreak signal)*

Adult man 35 years, fever 5 days, RDT negative, took coartem for 3 days but no improvement, headache and sore throat

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Live Lassa signal in state.*

Agree / **Disagree** Correction: _____

la-03 *(Edo)*

Woman 30 years, fever 6 days, vomiting and abdominal pain, took amoxicillin and antimalarial, no better, RDT negative

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Iba policy (pending clinician review, docs/CLINICAL_REVIEW.md §1): case definition met in an endemic state -> Refer now; partial features -> Refer within 24h.*

Agree / **Disagree** Correction: _____

la-04 *(Bauchi)*

Man 42 years, fever 4 days, bleeding from gums and nose, RDT negative

Gold triage level: **Refer now**

Danger signs: bleeding

Top differential: lassa

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Bleeding is also a danger sign.*

Agree / Disagree Correction: _____

la-05 *(Lagos)*

Trader 38 years returned from Owo in Ondo State 1 week ago, fever 5 days, sore throat, body weakness, RDT negative, no improvement on antimalarial

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Flagged as tricky: *non-endemic state, travel history*

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Criterion (c): travel to a high-burden area; rule does not cover travel, the model must.*

Agree / Disagree Correction: _____

la-06 *(Ebonyi)*

Man 50 years, fever 4 days, headache, took coartem, no better, RDT negative

Gold triage level: **Refer within 24h**

Danger signs: none

Top differential: lassa

Note: **WITH A LIVE OUTBREAK SIGNAL: Refer now (must refer: True)**

Flagged as tricky: *partial features tier*

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Iba policy (pending clinician review, docs/CLINICAL_REVIEW.md §1): case definition met in an endemic state -> Refer now; partial features -> Refer within 24h. No listed symptom -> partial features. With a live Lassa signal in the state, the live tier applies (fever >= 3 days with no treatment response) -> Refer now.*

Agree / Disagree Correction: _____

la-07 *(Ondo)*

Woman 27 years, fever 4 days, RDT positive, sore throat, vomiting, body weakness

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa, malaria

Flagged as tricky: *RDT-positive malaria with Lassa features*

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. RDT-positive does not exclude Lassa (co-infection). Iba policy (pending clinician review, docs/CLINICAL_REVIEW.md §1): case definition met in an endemic state -> Refer now; partial features -> Refer within 24h.*

Agree / Disagree Correction: _____

la-08 *(Taraba)*

Nurse 33 years, cared for a confirmed Lassa patient 10 days ago, now fever 3 days with muscle pain

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Criterion (b): contact with a confirmed case.*

Agree / **Disagree** Correction: _____

la-09 *(Plateau, live outbreak signal)*

Farmer 45 years, fever 5 days, generalised body weakness, took antimalarial, no change, RDT negative

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Live Lassa signal in state (Plateau not in baseline).*

Agree / **Disagree** Correction: _____

la-10 *(Ondo)*

Man 25 years, fever 1 day, cough and runny nose, RDT negative, eating well

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: viral, upper respiratory

Flagged as tricky: *endemic state, Lassa NOT suspected*

Guideline rationale: *Negative control: fever < 3 days and no listed symptom, so the NCDC case definition is not met. WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA).*

Agree / **Disagree** Correction: _____

Suspected meningitis

mn-01 *(Kano)*

Man 20 years, sudden fever and severe headache 1 day, neck is stiff, vomiting

Gold triage level: **Refer now**

Danger signs: none

Top differential: meningitis

Guideline rationale: *WHO IMCI (p.8): fever with stiff neck (or any general danger sign) = very severe febrile disease -> refer urgently. (NCDC CSM case management guide not yet indexed.)*

Agree / **Disagree** Correction: _____

mn-02 *(Sokoto)*

Baby 7 months, fever 1 day, bulging fontanelle, crying a lot, not breastfeeding

Gold triage level: **Refer now**

Danger signs: unable_to_drink_or_breastfeed

Top differential: meningitis

Guideline rationale: *WHO IMCI (p.8): fever with stiff neck (or any general danger sign) = very severe febrile disease -> refer urgently. (NCDC CSM case management guide not yet indexed.) WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

mn-03 *(Katsina)*

Woman 25 years, fever 2 days, headache, now confused and drowsy, RDT negative

Gold triage level: **Refer now**

Danger signs: lethargy_or_unconsciousness

Top differential: meningitis

Guideline rationale: *WHO IMCI (p.8): fever with stiff neck (or any general danger sign) = very severe febrile disease -> refer urgently. (NCDC CSM case management guide not yet indexed.)*

Agree / Disagree Correction: _____

mn-04 *(Zamfara)*

Child 5 years, fever and neck stiffness since yesterday, had a fit this morning

Gold triage level: **Refer now**

Danger signs: convulsions

Top differential: meningitis

Guideline rationale: *WHO IMCI (p.8): fever with stiff neck (or any general danger sign) = very severe febrile disease -> refer urgently. (NCDC CSM case management guide not yet indexed.) WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

mn-05 *(Jigawa)*

Boy 12 years, fever 1 day, stiff neck, purple rash on legs that does not fade

Gold triage level: **Refer now**

Danger signs: none

Top differential: meningitis

Guideline rationale: *WHO IMCI (p.8): fever with stiff neck (or any general danger sign) = very severe febrile disease -> refer urgently. (NCDC CSM case management guide not yet indexed.)*

Agree / Disagree Correction: _____

mn-06 *(Lagos)*

Man 30 years, fever 1 day, headache and neck pain after carrying a heavy load, neck moves freely, RDT positive

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Flagged as tricky: *neck pain without stiffness*

Guideline rationale: *Negative control: no neck stiffness on exam; RDT positive, no danger signs. WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / Disagree Correction: _____

Cholera / acute watery diarrhoea

ch-01 *(Borno)*

Man 30 years, profuse watery stool like rice water since morning, vomiting, sunken eyes, unable to drink, very weak

Gold triage level: **Refer now**

Danger signs: unable_to_drink_or_breastfeed

Top differential: cholera, acute watery diarrhoea, gastroenteritis

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A. Severe dehydration.*

Agree / Disagree Correction: _____

ch-02 *(Bauchi)*

Woman 45 years, many watery stools since yesterday, no fever, leg cramps, thirsty and drinking eagerly, skin pinch goes back slowly

Gold triage level: Refer within 24h

Danger signs: none

Top differential: cholera, acute watery diarrhoea, gastroenteritis

Flagged as tricky: *cholera without fever*

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A. Some dehydration; suspected cholera in a cholera-prone state needs notification and testing.*

Agree / Disagree Correction: _____

ch-03 *(Yobe)*

Child 3 years, watery diarrhoea 2 days, lethargic, sunken eyes, drinking poorly, fever 1 day

Gold triage level: Refer now

Danger signs: lethargy_or_unconsciousness

Top differential: cholera, acute watery diarrhoea, gastroenteritis

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A. WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): any general danger sign -> refer urgently.*

Agree / Disagree Correction: _____

ch-04 *(Adamawa)*

Man 60 years, watery diarrhoea and vomiting since morning, cannot stand, very sunken eyes, no fever

Gold triage level: Refer now

Danger signs: prostration

Top differential: cholera, acute watery diarrhoea, gastroenteritis

Flagged as tricky: *cholera without fever*

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A. Severe dehydration.*

Agree / Disagree Correction: _____

ch-05 *(Lagos)*

Child 2 years, loose stools 3 times today, fever 1 day, RDT negative, drinking normally, playful

Gold triage level: Treat & monitor

Danger signs: none

Top differential: diarrhoea, gastroenteritis

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A. No dehydration -> Plan A.*

Agree / Disagree Correction: _____

ch-06 *(Kano)*

Woman 38 years, watery stools 6 times since morning, mild fever, drinking well, alert, no sunken eyes

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: cholera, acute watery diarrhoea, gastroenteritis

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A. No dehydration; give ORS, notify if cholera suspected.*

Agree / Disagree Correction: _____

Uncomplicated malaria

um-01 *(Lagos)*

Adult woman 28 years, 62 kg, fever 2 days, RDT positive, eating and drinking well

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / Disagree Correction: _____

um-02 *(Oyo)*

Man 45 years, 80 kg, hot body and chills 1 day, headache, RDT positive, walking fine, no vomiting

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / Disagree Correction: _____

um-03 *(Kaduna)*

Child 6 years, 20 kg, fever 2 days, RDT positive, playing and eating, no vomiting, no convulsion

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists). WHO IMCI Chart Booklet (2014), general danger signs (p.2, p.8): none present.*

Agree / Disagree Correction: _____

um-04 *(Enugu)*

Teenage girl 15 years, 45 kg, fever and body pain 2 days, RDT positive, drinking well, alert

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / Disagree Correction: _____

um-05 *(FCT)*

Young man 22 years, 65 kg, fever since yesterday, mild headache, RDT positive, alert and walking

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / **Disagree** Correction: _____

um-06 *(Rivers)*

Child 3 years, 14 kg, hot body 1 day, RDT positive, breastfeeding and drinking well, active

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / **Disagree** Correction: _____

um-07 *(Anambra)*

Woman 35 years, 70 kg, not pregnant, fever 2 days, joint pains, RDT positive, eating well

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / **Disagree** Correction: _____

um-08 *(Kwara)*

Man 60 years, 72 kg, fever 1 day, weakness but walking, RDT positive, no confusion, passing normal urine

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Flagged as tricky: *older adult, 'weakness but walking' must not read as prostration*

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / **Disagree** Correction: _____

um-09 *(Delta)*

Boy 10 years, 30 kg, fever 2 days, RDT positive, no convulsions, not vomiting, drinking well

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Flagged as tricky: *negated danger signs must not trigger*

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / **Disagree** Correction: _____

um-10 *(Kano)*

Woman 50 years, 68 kg, fever 1 day, shivering, RDT positive, alert, eating

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / **Disagree** Correction: _____

Other febrile illness

ot-01 *(Lagos)*

Woman 35 years, fever every day for 9 days, abdominal pain, constipation, RDT negative

Gold triage level: **Refer within 24h**

Danger signs: none

Top differential: typhoid, enteric fever

Guideline rationale: *WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA).*

Agree / **Disagree** Correction: _____

ot-02 *(Lagos)*

Adult 30 years, fever 2 days, RDT negative, cough and runny nose, eating well

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: viral, upper respiratory

Guideline rationale: *WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA).*

Agree / **Disagree** Correction: _____

ot-03 *(Oyo)*

Woman 24 years, fever 2 days, burning when passing urine, going to toilet often, RDT negative

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: urinary tract infection, uti

Guideline rationale: *WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA).*

Agree / **Disagree** Correction: _____

ot-04 *(Enugu)*

Child 4 years, fever 2 days, ear pain and pus from the ear, RDT negative, playing

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: otitis, ear infection

Guideline rationale: *WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA).*

Agree / **Disagree** Correction: _____

ot-05 *(Kaduna)*

Man 45 years, fever and night sweats for 3 weeks, cough, weight loss, RDT negative

Gold triage level: **Refer within 24h**

Danger signs: none

Top differential: tuberculosis, tb

Guideline rationale: *WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA). Suspect TB.*

Agree / Disagree Correction: _____

ot-06 *(Kaduna)*

Man 29 years, fever 2 days, headache, RDT negative, alert, drinking well

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: viral, typhoid

Guideline rationale: *WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA). Test before treating; review in 3 days.*

Agree / Disagree Correction: _____

ot-07 *(Kano)*

Child 3 years, fever 3 days, generalised rash, cough, red eyes, drinking well, alert

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: measles

Guideline rationale: *WHO IMCI measles classification (p.8): measles without complications -> treat at home (vitamin A) and follow up.*

Agree / Disagree Correction: _____

ot-08 *(FCT)*

Woman 40 years, fever 3 days, cough with yellow sputum, chest pain when breathing in, RDT negative, walking, alert

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: pneumonia, chest infection

Guideline rationale: *No danger signs; community-acquired pneumonia without distress can be treated at PHC with review (clinician to confirm). WHO IMCI (p.8): if fever present every day for more than 7 days, refer for assessment; malaria test negative -> look for other cause (FEVER: NO MALARIA).*

Agree / Disagree Correction: _____

Nigerian Pidgin phrasing (cross-category)

pc-01 *(Lagos)*

Oga 30 years, body dey hot 2 days, RDT positive, e dey chop well, e dey waka

Gold triage level: **Treat & monitor**

Danger signs: none

Top differential: malaria, uncomplicated malaria

Flagged as tricky: *Pidgin*

Guideline rationale: *WHO Guidelines for Malaria: parasitologically confirmed malaria with no features of severe malaria is uncomplicated (p.169); test before treat (p.17). Manage at PHC with ACT and review (WHO IMCI p.8: follow-up in 3 days if fever persists).*

Agree / Disagree Correction: _____

pc-02 *(Oyo)*

Madam 25 years, belle 7 months, hot body 3 days, eye don yellow, piss dey black, RDT positive

Gold triage level: **Refer now**

Danger signs: jaundice, dark_urine

Top differential: severe malaria

Flagged as tricky: *Pidgin*

Guideline rationale: *WHO Guidelines for Malaria, severe malaria criteria (p.212: impaired consciousness, prostration, multiple convulsions, jaundice, bleeding, etc.) -> refer. Pregnant.*

Agree / **Disagree** Correction: _____

pc-03 *(Ondo)*

Man 40 years, body dey hot 6 days, throat dey pain am, e don take malaria drug no change, e dey vomit, RDT negative

Gold triage level: **Refer now**

Danger signs: none

Top differential: lassa

Flagged as tricky: *Pidgin*

Guideline rationale: *NCDC Lassa Fever Case Management Guideline (2018) §1.1.2 suspected case (pp.8-9): fever 3-21 days + >=1 of vomiting, diarrhoea, sore throat, myalgia, generalised weakness, abnormal bleeding, abdominal pain; index of suspicion raised by no response to antimalarials/antibiotics, contact, travel to high-burden area. §2.1.1 (p.11): holding area, IPC, notify. Iba policy (pending clinician review, docs/CLINICAL_REVIEW.md §1): case definition met in an endemic state -> Refer now; partial features -> Refer within 24h.*

Agree / **Disagree** Correction: _____

pc-04 *(Borno)*

Mama 40 years, stool dey run like water since morning, e dey vomit, body don weak, e no fit stand

Gold triage level: **Refer now**

Danger signs: prostration

Top differential: cholera, acute watery diarrhoea, gastroenteritis

Flagged as tricky: *Pidgin, cholera without fever*

Guideline rationale: *NCDC AWD/Cholera Outbreak Guideline, suspected case definition (p.12); WHO IMCI dehydration classification (p.7): severe dehydration -> Plan C / refer urgently; some -> Plan B; none -> Plan A.*

Agree / **Disagree** Correction: _____